

- Opioids cross the placenta easily and pass into the baby's bloodstream. This can have the affect of sedating the baby and can affect the baby's breathing after the delivery. The administration of opioid medications therefore needs to be carefully timed during labor so that they are not given too close to the time of the delivery of the baby.
- Opioids can depress the mother's breathing during labor. However, this side effect is more common when the mother has a preexisting respiratory illness, such as asthma or emphysema.
- Opioids cause a delay in emptying the bowels and therefore increase the overall risk to the mother should general anesthesia be needed later on (see [Pudendal blocks](#)).

Tranquilizers

These drugs don't relieve pain, but they can help to relieve anxiety and help a woman relax for several hours during early labor. Some women who are very anxious about the impending pain or of labor and childbirth may seek this sort of medication.

Tranquilizers may be given orally, intravenously, or injected into a large muscle. Oral dosage takes the longest to take effect. Some women dislike what tranquilizers do to them, since they may feel drowsy and out of control. If the dose is very high, women may nod off between contractions. It may be hard for a woman to fully remember her labor experience if she's taken tranquilizers. The drugs also affect the baby, decreasing his activity and muscle tone during and after birth. For these reasons, tranquilizers are not for everyone, but in certain circumstances, they can help a woman feel less anxious about labor.

ASPIRIN: NOT FOR LABOR PAIN

When you start to experience the pain of labor contractions at home, no matter how tempting it may be, do not reach for an aspirin to relieve your pain. Aspirin can be extremely dangerous during pregnancy, particularly in the third trimester and leading up to labor and delivery. The drug interferes with your blood-clotting ability, which means that your risk of a hemorrhage during delivery increases if you take aspirin in the weeks leading up to your baby's birth. Taking too much aspirin in the last trimester may actually prolong labor. Bleeding problems may even develop in your newborn. Taking aspirin regularly toward the end of your pregnancy can also affect your baby's heart or blood flow, so it's best to avoid aspirin, unless your doctor recommends you take it. Your best bet for dealing with labor pain at home is to keep active until it's time to head to the hospital. While you wait, do something to distract yourself from the pain, whether it's counting the length of time between contractions, packing your bag for the hospital, or anything else that feels right. Ask your partner for a massage or try a warm shower. No matter what you do, refrain from drug use. This way, if you need pain relief when you get to the hospital—many women do—you won't have any medication in your system.

Epidurals

An epidural is a regional anesthetic that can be given at any stage of labor to numb the abdomen and therefore block the pain of contractions, but it's typically given when the woman is at least 4 cm dilated. In some cases, if a woman's labor progresses very quickly and she doesn't request one right away, she may not be able to get one before delivery. Epidurals are the most popular form of pain relief during labor in the US, with more than half of pregnant women delivering in hospitals choosing this treatment.

How epidurals work A hollow needle is inserted between two vertebrae in the lower back. A tiny plastic tube is then passed through the needle and into the epidural space